



A SOOTHEMADE BOOK

Birth Is Scary

A field guide to the fear of giving birth.

Soothemade

A SMALL APOTHECARY · MADE SLOWLY

Dedication

For the pregnant person at week thirty-five, terrified in a way she has not told anyone.

The fear is data. The fear is not a forecast.

A note from Maya

You are afraid of giving birth.

You have been told to "trust your body." You have been told "women have done this for thousands of years." You have been told the fear is irrational. None of those have helped, because the fear is not irrational. Birth is a real event with real intensity and real risks. The fear is your nervous system paying attention to what is coming.

This book treats the fear seriously. It names what you are actually afraid of, in specific. It gives tools that are not platitudes. It gives you scripts for the conversations you need to have, with your provider, with your partner, with yourself.

It does not promise that the birth will go your way. No book can. What it does is give the fear somewhere to go besides the pit of your stomach at 3am.

With care, Maya

This book is not therapy. If your birth anxiety is interfering with eating, sleeping, daily function, or your bond with the pregnancy, please see a perinatal mental-health therapist. They specialize in exactly this. Crisis lines are at the back of the book.

Chapter 1: Birth IS Scary

This chapter is permission.

You have been told things that did not help. "Don't worry about it." "It'll be fine." "Trust your body." "Women have been doing this for thousands of years." "Your body knows what to do." "It's a beautiful experience." "Don't psych yourself out."

None of these acknowledge the truth.

The truth is: birth is an intense physical event. It is one of the most physically demanding things a human body does. Many people describe the pain as the most intense they have ever experienced. Some experience trauma during it. Some experience complications. Some experience joy. Sometimes all of those in the same labor.

Modern obstetric care has reduced the risks dramatically over the last century. It has not eliminated them. The risks for you depend on your specific situation, your specific health history, and which team you give birth with.

You are allowed to be scared. The fear is data. It is your nervous system telling you that something big is coming. You can hold the fear and prepare for the birth at the same time.

The opposite of the "just relax" advice is not "be paralyzed by fear." It is "be informed. Be clear about what you want. Be prepared to ask for what you need. Have people who will advocate for you."

That is the work this book helps with.

A small principle before we go further: anxiety hates specificity. "I am afraid of birth" is a fog. "I am afraid of being unable to communicate during transition because I have read that some people can't" is a specific worry that can be talked about. Specificity lets the fear be addressed.

The next six chapters name the most common specific fears. Read the ones that resonate. Skip the ones that do not. The fears you carry are the ones to work on.

Chapter 2: Fear of Pain

The most common one. Also the most under-discussed.

What pregnant people are actually afraid of, when they say they are afraid of the pain:

- That it will be more than they can handle.
- That they will not be able to communicate through it.
- That they will lose themselves in it.
- That they will have to ask for relief, and not get it.
- That the relief will not work.
- That the medications used will harm the baby or themselves.

Each of these is a real fear. Each has a real response.

The pain-relief menu in modern hospitals is wider than most pregnant people know. Movement and position changes. Water (showers, tubs). Breathing. Counter-pressure from a doula or partner. IV medications. Epidural. Nitrous oxide in some hospitals. Combinations of all of the above.

The right option for you is the one you and the team choose together. You can change your mind during labor. You can ask for pain relief at any point. You do not need to "earn" it by suffering first.

Many people who planned an unmedicated birth choose pain relief during labor. Many who planned an epidural end up not getting one. Either way is valid. The version of you that arrives at labor will have information the version of you that wrote a birth plan did not have. Listen to her.

Tools to bring with you (in addition to whatever pharmacological options you choose):

- A doula trained in labor support.
- A partner who knows your preferences.
- A playlist (or specific music you want).
- Specific lighting (dim, candles if allowed, no fluorescents).
- A focal point or object.
- A scent (essential oil, if allowed, that is yours).
- A blanket from home.
- A picture of someone or somewhere that grounds you.
- Practiced breathing technique.
- Position changes the team supports.

Practice beforehand. The "tools" only work if you have practiced them. The breath you did once in a yoga class is not the breath that will help you in labor. Either practice a specific technique you have learned, or release the idea that breathing alone will do the work.

The thing to tell your partner about pain: "You are allowed to ask for relief on my behalf. You do not have to wait for me to ask. If you see me at the threshold, ask."

The version of you in labor may not have the words. The version of your partner standing next to you can.

Chapter 3: Fear of Loss of Control

The second most common fear. The one that is hardest to name out loud.

What it looks like, specifically:

- Being unable to think clearly.
- Being unable to speak for yourself.
- Being pressured into a decision quickly.
- Strangers entering the room without warning.
- Being told to do something you do not want to do.
- Being too tired to advocate for yourself.
- Being naked or exposed in front of people you do not know.

There are things you can control in advance. Who is in the room (partner, doula, family, support person). Who is NOT in the room (with the team's understanding). A birth plan that names your preferences in writing. A doula whose job is partly to advocate. Practiced statements you can use even when tired. A code phrase between you and your partner for "stop and check in with me, do not let them rush me."

A reminder about what you can ask for (and what your partner can ask for on your behalf):

- "I'd like a moment to think about this."
- "Can you explain what happens if we wait."
- "I want my partner to be in this conversation."
- "I am not consenting to that right now."

- "I want to talk with the [doula / patient advocate / charge nurse]."

You can ask the team to pause and explain anything they propose. To give you a minute before a decision. To bring in your partner before a non-emergency decision. To tell you what each thing they are doing is for. To lower their voices. To knock before entering. To limit the number of people in the room. To use specific words you prefer (and avoid words you do not).

You can decline routine interventions you have explicitly opted out of in your birth plan, in the absence of medical emergency. Specific staff being in the room (within reason). Procedures that are non-emergency and you have not consented to.

You can request a second opinion before a non-emergency decision. Reading time before signing a non-emergency consent. A specific provider when possible.

A note on emergencies. In a true emergency, you may not have time for all of the above. That is real. What the team is doing in those moments is often the right thing. The fear here is not about emergencies. It is about the non-emergency moments where you should have had time and were not given it. Those moments are the ones to be ready for.

Chapter 4: Fear of Complications for You

This is the fear most people are most afraid to name.

The fear of dying. The fear of having a serious complication. The fear of hemorrhage, of preeclampsia, of a long recovery, of permanent injury (incontinence, prolapse, scars).

Naming the specific complication you are afraid of is the first move. Then knowing whether it is something the team is watching for in your specific situation.

What to ask your provider: what is my specific risk profile, what are the early-warning signs you are watching for, what would prompt you to recommend an intervention, what is the escalation path if something develops.

What to watch for yourself (and when to call before labor even starts): sudden severe headache, vision changes, sudden swelling (especially face and hands), severe upper-right belly pain, bleeding, decreased baby movement, or anything else your provider has named for your specific situation.

A reminder about the statistics. Statistics are about populations, not about you specifically. "The risk is 1 in X" is not the same as your individual risk. Your provider's read of your situation is what matters. Statistics that scare you in the wrong direction are also worth talking through with your provider. Sometimes the internet's number is wrong for you.

Modern obstetric care has greatly reduced maternal mortality from where it was a century ago. The reduction has

been uneven across demographics and locations, and your specific risk profile matters. If you are at higher risk (your provider knows your specifics, but common factors include certain chronic conditions, prior pregnancy complications, certain demographic factors), your team will be more attentive, and the right move is to have a higher-care setting, more frequent monitoring, and clear escalation pathways.

Who else needs to know your medical history in the room:

- Partner (knows what to say if you cannot).
 - Doula.
 - A document in your hospital bag with key history.
 - Emergency contact who can speak on your behalf.
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Chapter 5: Fear of Complications for the Baby

The fear most pregnant people hold quietly. The hardest to share.

When this fear is loudest: at night when the baby has been quiet, after a scary story online, around the gestational age of a prior loss, approaching the appointment, during labor itself.

What to do when the fear is loudest:

1. Note what you have noticed about the baby (or not noticed). Decreased movement? Cramping? Bleeding? Anything physical?
2. If anything physical is concerning, call the provider's after-hours line. This is what it exists for.
3. If nothing physical is happening, the fear is the thing. The fear can be sat with. The fear is not a prediction.
4. Do not search online. Search makes it worse.
5. Call a person who knows you. Partner, friend, doula.

Kick counts (if recommended for you). When your provider has you do kick counts, follow the specific instruction. Know what "decreased movement" means in your own words. Save the phone number you will call if you are unsure.

What to prepare, if something does go wrong. This is not pessimism. This is planning. Knowing that books and resources exist for loss is itself a form of permission to feel the fear without it owning you.

Who you would call first. Who you would tell second. What you would want, right then.

The truth about the fear. Most pregnancies result in a healthy baby. Most concerning moments during pregnancy turn out to be okay. Most monitoring findings that prompt closer watch resolve into a normal birth.

Some don't. That is the part the fear is responding to. The fear is not stupid. It is the cost of loving someone you have not met yet.

What helps: trust the provider's monitoring (they are watching for the things you are afraid of). Know the signs that warrant a call. Have a routine for when the fear hits at 3am: a glass of water, a hand on the bump, paying attention to movement, a slow exhale. If no movement, call the line. Limit the inputs. Stop reading stories about loss in the middle of the night. The brain stores them and brings them back later.

A reminder: the fear does not make the feared thing more likely. The fear is a feeling. The feeling is real. The feeling is not a prediction.

Chapter 6: Fear of Interventions You Did Not Want

For the pregnant person whose birth plan is leaning a specific direction and who is afraid of having the plan overridden.

The interventions most commonly feared: induction when planning a spontaneous labor, synthetic labor augmentation (Pitocin), breaking water artificially, continuous fetal monitoring (instead of intermittent), epidural when planning without, forceps or vacuum assistance, episiotomy, c-section when planning a vaginal birth, separation from the baby after birth, routine procedures you have not consented to.

Preparation moves:

- Write the birth plan. State preferences clearly.
- Talk to the provider about your preferences in advance, not in labor.
- Ask about the hospital's rates (c-section, episiotomy, induction) for context.
- Identify the specific interventions you most want to avoid and the specific situations where you would still accept them.
- Discuss with your partner what you want them to do if you are too tired to advocate.

The hard truth. Sometimes interventions you did not want are the right call. The team is making a decision based on what they see in real time. An emergency c-section that saves your baby is not the same as a "convenience c-section." A repair of significant

tearing is the right move. Pitocin to manage stalled labor sometimes prevents a worse intervention later.

The fear is not about the interventions themselves. It is about having interventions PUSHED on you that were not necessary, or about not understanding what is happening to your body in real time.

What to ask in the moment:

- "Is this an emergency or do we have time to talk?"
- "What happens if we wait 30 minutes / 1 hour?"
- "What are you seeing that is making you suggest this?"
- "Can we try [alternative] first?"

What to agree on in advance, as a couple:

- The interventions you will accept without question in an emergency.
- The interventions you want to talk about before agreeing to.
- A "code phrase" between you for "I need you to slow this down with the team."

A note: after the birth, if you felt pushed into interventions that you did not want, that is a real feeling and can be processed. Birth trauma is real. Perinatal therapists help with it.

Chapter 7: Fear of the Unknown

For the first-time parent. For the parent whose last birth was traumatic. For anyone whose fear does not fit a category.

What the unknown feels like: I have never given birth before. I have given birth and it was traumatic. I have given birth and it was different from what I expected. I have read so much that the unknown feels both bigger and stranger. I cannot picture how it will go.

What you can pre-view:

- A hospital tour (most hospitals offer them).
- A childbirth education class.
- A doula consultation.
- A conversation with a friend who has given birth.
- Reading a couple of birth stories from people whose values match yours (avoid the trauma-spiral side of the internet).

What you cannot pre-view:

- The exact day labor starts.
- How long it will be.
- What it will feel like for you.
- What your body will do.
- Who will be the on-call provider.

These are unknowable in advance. The work is making peace with the unknowable, not erasing it.

Tools for living with the unknown.

Prepare what you can. The bag is packed. The hospital is known. The route is known. The backup route is known. The parking situation is known. The team is told where you are giving birth. The pediatrician is chosen. The first 48 hours after the baby comes home has a plan.

Let go of what you cannot. The exact day. The duration. The pain level. Who is on call. The exact path. The plan will adjust.

Build a "whatever happens" mantra. A single sentence that grounds you when the fear of the unknown is loud. Find yours.

A word about birth stories. Reading birth stories can be helpful when the stories you read are from people whose values and outcomes match yours, told with honesty without dramatizing.

Reading birth stories can be harmful when they are skewed toward the dramatic, the traumatic, or the "I knew something was wrong" genre.

For this stretch of pregnancy, consider a limit on how much birth content you take in. The "I read every story I could find" approach is rarely the calming one.

Chapter 8: Scripts for Telling the Team You Are Scared

Most pregnant people do not tell their providers how scared they are.

This chapter is the words.

At an appointment:

I have been more anxious about the birth than I expected. I want to tell you some of the specific fears I have, so we can talk about each one before labor.

Before a specific procedure or discussion:

I am nervous about this. Can you walk me through it slowly and tell me what is normal vs what I should worry about.

Asking for a referral:

I think my anxiety is bigger than what we can address in appointments. Can you refer me to a perinatal therapist?

When a nurse or doctor dismisses your fear:

I hear you that statistically it's unlikely. I am asking you to take this concern seriously anyway.

Asking to be believed:

I have a history of [trauma / prior loss / dismissal in medical settings]. I am telling you so you can take that into account in how you talk with me.

At the hospital arrival:

I am scared. I want the team to know that. I am asking you to be patient with me during this labor.

For your partner to use on your behalf:

She is afraid of [specific thing]. I am asking you to take that seriously when you talk with her about [decision].

A note: telling the team you are scared is not weakness. It is informed consent. Providers who listen to fear deliver better care. Providers who dismiss it are a different question you can answer before labor.

Chapter 9: The Night Before Labor

For the version of you on the eve of induction, or the version of you in early labor at home.

This chapter is short on purpose. The night before labor is not the night to read a long book.

What to do tonight:

- Eat something even if you are not hungry.
- Drink water.
- Take a warm shower.
- Lay out clothes.
- Charge the phone.
- Set the alarm.
- Hug your partner.

What you have already packed: the bag, the birth plan (printed), this book (in the bag if you find it helpful), phone fully charged, snacks for partner, going-home clothes for you and baby, car seat installed.

A note from now, to you in labor:

When you read this, you are scared. That is fine. The fear is information, not prediction. You have prepared. You have a team. You have me on your side, from before this. You are not alone.

This is the last night of being just you. Tomorrow you will have done something enormous.

Tonight, sleep.

Chapter 10: If You Are Not Okay

Birth anxiety has a range. At one end, the worry is uncomfortable but manageable. At the other end, it is consuming. The book helps the middle. The far end is for a perinatal mental-health professional.

The honest check:

- Is the anxiety interfering with sleeping?
- Is it interfering with eating?
- Is it making me avoid prenatal care?
- Am I having panic attacks?
- Am I having intrusive thoughts about harm?
- Am I unable to think about anything else?
- Have I had nightmares about the birth?
- Am I considering avoiding the birth somehow?

If two or more of these are true, the next step is a perinatal mental-health professional. Not the book. Not your provider alone. A specialist.

Perinatal anxiety and depression are treatable. The treatment is good. The sooner you get to it, the faster the anxiety becomes manageable. Some treatment is talk-only. Some includes medication that is okayed by your OB and your prescriber together. Both are valid.

A find-a-therapist checklist:

- Ask your OB for a referral.

- Search Postpartum Support International's provider directory.
- Check your insurance's mental-health network.
- Look for "perinatal" or "reproductive mental health" specialty.
- A consultation call before committing is reasonable.

A final reminder: seeing a therapist is not failure. Some of the calmest people in labor are the ones who have been in perinatal therapy for months beforehand. The preparation is the gift.

Chapter 11: A Final Letter

You did the work of naming what you are afraid of.

Most pregnant people who are afraid of birth do not name it that specifically. They carry it around as a fog. You sat down with this book and made it specific. That is the harder thing. The fog cannot be addressed. The specific fear can.

Whatever happens during the birth, you are not the version of you who picked up this book and refused to do anything about the fear. You did something about it. You named it. You brought it to the people who could help. You wrote scripts. You made a plan.

You may still be afraid in labor. That is allowed. You will also be capable. Both can be true.

The labor will end. The baby will arrive. The fear, for the most part, lifts in the days after, replaced by other things. Some of the fear is the thing you carried. Some of it was preparation. Both are forms of love.

You are not alone in this, even when the room is full of strangers.

With care, Maya

About Soothemade

Soothemade is a small apothecary of printables, planners, cards, and books for new parents.

The printable companion to this book is the **Birth Anxiety Workbook** at notes.soothemade.com, with fear-inventory pages and scripts you can write on.

Also from Soothemade

- **The Birth Plan Pack**, the practical preferences document.
 - **The VBAC Prep Planner**, if you are facing the specific VBAC fear.
 - **A Rainbow Baby Planner**, when prior loss is part of the anxiety.
 - **The Postpartum Anxiety + PPD Journal**, for after.
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Crisis lines

If thoughts of harm to yourself or the baby are present, call.

United States

- 988, Suicide and Crisis Lifeline (call or text)
- 1-833-852-6262, Postpartum Support International HelpLine
- 1-833-943-5746, Maternal Mental Health Hotline (24/7)

United Kingdom

- 116 123, Samaritans

Canada

- 9-8-8, Suicide Crisis Helpline

Australia

- 1300 726 306, PANDA

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